Which treatment this guide covers This guide covers lorlatinib alone for ALK-positive advanced non-small-cell lung cancer. The available Irish National Cancer Control Programme (NCCP) v3 document lists use after particular earlier ALK medicines. The current European licence also includes people who have not had an ALK inhibitor. The team must confirm your treatment setting and the current Irish protocol and access arrangements before prescribing; the source versions currently disagree.

NCCP 00570 · VERSION 3

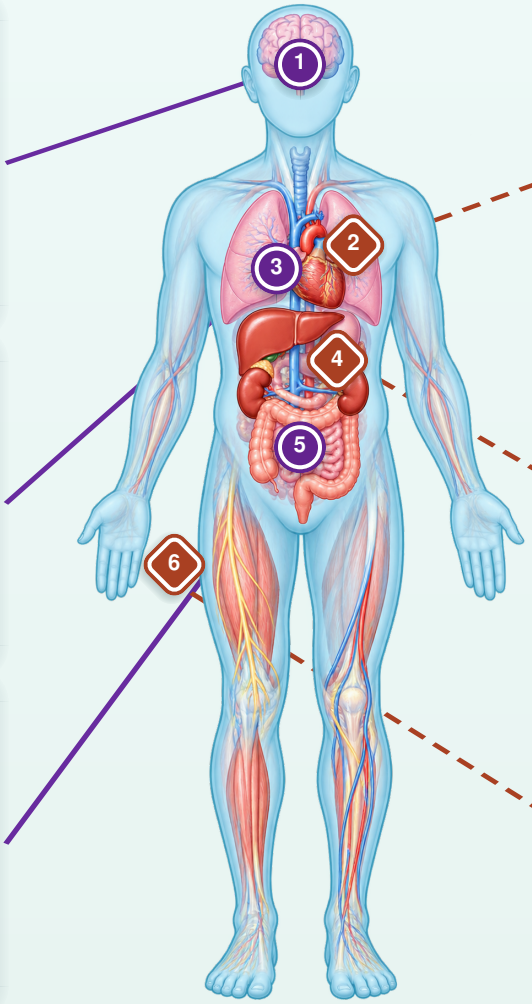
Lorlatinib therapy

Lorlatinib is taken once daily continuously, with no planned rest week. Treatment continues while it is helping and side effects remain acceptable; there is no fixed number of treatment cycles.

ALK-targeted tablets · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Thinking / mood / speech

Lorlatinib

Report changes in memory, behaviour, mood or perceptions promptly.

2 Lungs / breathing

Lorlatinib

New or worsening cough and breathlessness need urgent review.

3 Heart / blood pressure

Lorlatinib

Report fainting, dizziness, an irregular heartbeat or new swelling.

4 Blood fats / sugar

Lorlatinib

These can rise without symptoms; attend scheduled blood tests.

5 Abdomen / pancreas

Lorlatinib

Report severe or persistent abdominal pain, nausea or vomiting.

6 Hands / feet

Lorlatinib

Report numbness, tingling, weakness or changes in balance.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact your oncology team promptly for changes in memory, thinking, mood, speech or behaviour, hallucinations, new or worsening breathlessness, cough, fainting, a slow or irregular heartbeat, severe abdominal pain, marked thirst or frequent urination. Severe confusion, unsafe behaviour, severe breathing difficulty or collapse needs emergency help. Do not wait for the next appointment. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

Know the next step.

Taking treatment

Lorlatinib is taken once daily continuously, with no planned rest week. Treatment continues while it is helping and side effects remain acceptable; there is no fixed number of treatment cycles.

The usual starting dose is 100 mg once daily; use the dose on your prescription. Take it at about the same time each day, with or without food. Swallow whole; do not crush, chew or split tablets, or use damaged tablets. Avoid grapefruit and grapefruit juice.

Checks and follow-up

Before starting: ALK testing, blood count, kidney and liver tests, clotting tests, cholesterol and triglycerides, glucose, pancreatic enzymes, blood pressure and an electrocardiogram (ECG). Lipids are checked at weeks 2, 4 and 8, then as needed. Blood count, kidney/liver tests, ECG and pancreatic enzymes are normally checked monthly. Blood pressure is checked after two weeks and at least monthly; glucose as needed. Scans follow your oncology plan.

The whole-treatment map shows the checks for this exact treatment setting.

Support between reviews

Taking and replacing tablets

If a dose is missed, take it when remembered if the next dose is at least four hours away; otherwise skip it. Do not take two doses together. If you vomit after a dose, do not take another tablet; take the next dose at the usual time. These instructions follow the current product leaflet.

Cholesterol and other medicines

Cholesterol or triglycerides may rise without symptoms. Attend blood tests and take any prescribed lipid-lowering medicine. Ask the pharmacist before starting or stopping any medicine, supplement or herbal product, including St John’s wort. Do not change the lorlatinib dose yourself.

Sickness and daily wellbeing

This oral regimen has minimal to low sickness risk. Use anti-sickness medicine only according to your own plan and report persistent vomiting or poor intake. A standard chemotherapy steroid or anti-sickness course is not automatically needed.

Thinking, mood and practical support

Ask someone you trust to help notice changes in thinking, mood, speech or behaviour, if you wish. Report changes early. Lorlatinib can affect driving and machine use, especially when central nervous system (CNS) effects are present; discuss whether it is safe for you.

Pregnancy, fertility and feeding

Lorlatinib may harm an unborn baby and make hormonal contraception less effective. Women who could become pregnant need highly effective non-hormonal contraception during treatment and for at least 35 days afterwards; discuss any unavoidable hormonal method and barrier protection. Men with partners who could become pregnant need effective contraception including a condom during treatment and for at least 14 weeks afterwards; condoms are also needed with a pregnant partner. Discuss fertility preservation before treatment. Do not breastfeed during treatment; agree when it can safely resume with the team.

Oncology daytime: Insert numbers here. **Out of hours:** Insert numbers here. Follow your own hospital’s urgent-care plan. Emergency: 112 / 999.



Your complete treatment reference

https://sactcheck.com/patient/00570/

Public information only. No personal information is encoded.